



OFFICIAL

Safe and Supportive Observations Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Mental Health Service	Mental Health Service	Medical, Nursing, Allied Health

2. Purpose

Mental Health staff are to promote a least restrictive, safe and supportive environment by determining a patient's clinical observation category by incorporating risk assessment, collaborative care and documentation.

3. Background

Collaborative care through consultation with the patient and the treating team is the basis for establishing a therapeutic relationship for quality safe and supportive care. Mental health staff always have a duty of care to maintain the safety and well-being of patients.

Clinical safe and supportive observation (SASO) forms a key intervention in the delivery of safe and therapeutic mental health care.

The observation of a patient is one intervention that may help to maintain their safety. This should be one of a range of interventions which all contribute to effective risk management.

A comprehensive risk assessment is undertaken for all mental health patients to identify their clinical need, mental state, behaviour and social and safety needs. The degree of concern regarding a patient's safety will determine how care is coordinated.

Decisions will be influenced by

- Severity of the patient's mental and physical illness
- The patient's level of compliance with treatment
- Risk of absconding
- The patient's level of impulsivity
- Presence of risks such as active suicidal ideation, sexual safety, exploitation, or safeguarding issues

The SASO category charts are the tools used to document visual contact and continuing risk assessment during admission. The observation category is determined for every patient and is increased when:

- there is information to indicate that the patient's risk has increased
- there is documented and or verbal/visual information to indicate the patient is at risk

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Close clinical observation is assessed by clinicians as necessary for treatment and/or it is agreed that the patient needs additional support

4. Procedure

- A formal Risk Assessment and Management Plan (RAMP) and Safe and Supportive Observation Chart is completed for all admissions.
- All consumers admitted to the Mental Health inpatient setting will be maintained at a minimum Category 4 (60-minute SASO) throughout their period of hospitalisation.
- After the initial risk assessment process, the determination of observation category for all patients is based on:
 - Treating team/psychiatrist observation and clinical assessment,
 - Nursing staff observation and clinical assessment,
 - Risk assessment documentation (Risk Assessment and Management Plan) and information provided by the referring source or significant others.
- Determination of observation category occurs by collaboration between all Mental Health practitioners.
- The patient and allocated nurse are notified of observation category.
- Ongoing risk assessment documentation is completed, including:
 - Visual observation checks recorded on the SASO category sighting charts include:
 - Time of sighting patient,
 - Patient's location,
 - Patient's activity – movement, if asleep checking for signs of life and documenting respiration rate, rise and fall of chest; if snoring, consider waking,
 - Staff initial progress notes in the patient's health record. These are to be completed at the end of every shift and details of the current category of observations and justification for any changes to category observations made,
 - Any deterioration in patient's condition is escalated to the treating team psychiatrist for changes in treatment regimens, medications and SASO observation categories.
- Identify, Situation, Observations, Background, Assessment/Action, Responsibility/Read back (ISOBAR) handover includes patient risk assessment and clinical observation category.
- Nursing staff may implement a higher observation category based on a change in the patient's clinical condition. Any increase in SASO category observations require the treating team to be informed and risk assessment documented in the patient's health care record.
- A decrease to a lower category observation only occurs with the collaboration of the treating team. The decision made is documented in the patient's health care record by the treating Medical Officer.
- Patients on SASO Category 1, 2 and 3 observations are required to be re- assessed daily by the treating team.
- Times of observations are not written in advance and are accurate and prompt.
- SASO categories are documented in patient health care record progress notes on each shift.
- Any changes to patients SASO levels to be recorded on the SASO Level Change sticker and placed in the health care record.

- Concerns over patient deterioration are escalated to shift coordinator, NUM/ANUM/AHCNS and treating team Medical Officer.
- Any change in SASO category observations requires patient leave status to be reviewed and communicated to the patient to advise of change.
- The need for either extra nursing staff or the use of existing resources on any shift will be determined by the Shift Coordinator after consultation with the NUM, ANUM or AHCNS. If the risk is assessed as high or extreme, extra nursing resources may be required.
- Minors admitted to the ward will require a 1:1 nurse special for the duration of the admission.
- AIN staff are not to be utilised in Yorgum.
- Sexual Vulnerability:
 - Unless clinically contraindicated, female patients deemed at risk are to be allocated to female nursing staff each shift and male patients deemed at risk are to be allocated to male nursing staff.
 - To ensure maximum visibility, patients deemed at risk are to be allocated to the following bedrooms:

Moodjar	M1 & M2	Male or female but must have patients of the same gender in both rooms.
Moodjar	M3A & M3B	Male or female. Double room, patients of the same gender only.
Moodjar	M4	Male or female
Moodjar	M5	Male or female
Yorgum	Y5	Female
Yorgum	Y6	Female
Karri Ward	K1	Male or female
Banksia	B1	Male or female

- Education should be provided to the patient on safe sexual practices as appropriate
- Two staff, at least one of the same genders as the patient, are to be present during all safe and supportive observations, throughout the night shift.
- If an indiscretion is identified, staff should follow the guidelines outlined in:
 - [Responding to Allegations of Sexual Assault, Reported Sexual Contact Procedure](#)
- The Treatment, Support, and Discharge Plan should be regularly updated in consultation with the patient and reflect the identified risk and SASO observation category

4.1 Courtyard supervision

- The courtyard must be searched prior to it being opened for patient access and checked to ensure that exits are secured, debris or unwanted items are removed and that potentially

harmful items have not been thrown over the external perimeter fencing or hidden in concealed areas (eg, drugs, alcohol, cigarettes, lighters, weapons).

- Staff supervising courtyards must carry their personal duress devices
- The level of supervision required for patients utilising courtyards will be determined by the shift coordinator and will be based on patient acuity. This supervision should be performed at least every 60 minutes.

5. Legislative context

The following documents are required to give effect to this procedure:

- Mental Health Act 2014
- Chief Psychiatrist of Western Australia
 - [Reporting Notifiable Incidents – Public Mental Health Services](#)
 - [Chief Psychiatrist's Sexual Safety Guidelines | Chief Psychiatrist | Government of Western Australia](#)
- HDWA
 - [Clinical Handover Policy MP 0095](#)
 - [Best Practice Guideline: Mental Health Consumers Who Are Missing or Absent Without Leave](#)
 - [Complaints Management Policy MP 0130/20](#)
- EMHS
 - [Mental Health Advocates](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Responding to Allegations of Sexual Assault, Reported Sexual Contact Procedure](#)
- [Communicating for Safety Procedure](#)

7. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated by Mental Health Safety and Quality Departmental Meeting. Monitoring activities will include:

- Monthly auditing
- Clinical incident reported data via Datix Clinical Incident Management System (CIMS).

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.5 Collaboration and teamwork
- Standard 5 – Action 5.10 Screening of risk
- Standard 6 – Action 6.3 Partnering with consumers
- Standard 6 – Action 6.4 Organisational processes to support effective communication

[Chief Psychiatrists Standards for Clinical Care](#)

[Chief Psychiatrists Guidelines for the Sexual Safety of Consumers of Mental Health Services](#)

National Standards for Mental Health Services

- Criteria 2.11 - The MHS conducts risk assessment of consumers throughout all stages of the care continuum, including consumers who are being formally discharged from the service, exiting the service temporarily and/or are transferred to another service.
- Criteria 9.2 - The MHS has formal processes to support and sustain interdisciplinary care teams.

9. Bibliography

1. Gale Academic One file: Safe and supportive observation in practice: a clinical governance project
<https://go.gale.com/ps/i.do?id=GALE%7CA132679413&sid=googleScholar&v=2.1&it=r&linkaccess=abs&issn=14658720&p=AONE&sw=w&userGroupName=anon%7E8c2ed1fa>
2. Special and Supportive Observations Policy: Royal Perth Bentley Group.
<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/RPH/Special%20and%20Supportive%20Observations%20Policy.pdf>
3. Observations: Safe and Supportive (Mental Health) Procedure. Rockingham Peel group
<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/SMAHS/RKPG/CP.MH.VisualObservationsMentalHealth.pdf>

10. Policy owner

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

Title: Nursing Coordinator
Department: Mental Health
Email: Dene.Olive@helath.wa.gov.au.

11. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	25 11 2025	25 11 2028	Full review, Minor amendments; room number update.

12. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott - Service Director Mental Health.
Authorisation date	25 11 2025
Published date	25 11 2025

13. Appendices

13.1 Appendix 1: Category Observations Risk Assessment Process

CATEGORY 1

1:1 Special within arm's reach

Restricted to the ward - Extremely High Risk - MAY BE REFERRED OR UNDER MHA2014

Extremely high level of risk of harm to self or others, acute diagnostic symptoms, suicidal and unable to contract safety, lack of insight and poor response to treatment, non-responsive to boundaries and limitations, non-compliant with medication regimen, no impulse control, unable to anticipate consequence, serious/continuous self-harm attempts, physical status continues to deteriorate and requires constant monitoring.

CATEGORY 2

1: 1 Special within eyesight

Restricted to the ward – High Risk - MAY BE REFERRED OR UNDER MHA2014

High risk of harm to self or others, high risk of absconding, possibility of escalation in diagnostic symptoms requiring increase of category / MHA or HDU, lack of insight and poor response to treatment, non-compliant with medication regimen, poor impulse control – unable to anticipate consequences, continuous self-harm attempts, escalating suicidal thoughts and/or plan but can contract to safety, current condition results in closer management of vulnerability, physical deterioration requiring closer monitoring medical attention.

CATEGORY 3

Increased Intermittent sightings. (15 or 30 minutes)

Leave is at discretion of Treating team and is documented in the progress notes

Medium risk of harm to self or others, possibility of escalation in diagnostic symptoms requiring increase in category. Poor insight and poor/unknown response to treatment, may test boundaries/limitations, may require negotiation for treatment/medication regimen, may show poor impulse control, self-harm behaviour may be present, escalating suicidal thoughts and/or plan but can contract to safety, may have physical deterioration requiring medical attention.

CATEGORY 4

60 Minute Sightings

Leave is at discretion of Treating team and is documented in the progress notes

Patient has partial insight and is responsive to treatment, compliant with group attendance, no impulsive behaviour, ability to control self, may have identifiable vulnerabilities in clinical setting, may have suicidal thoughts but no plan or intent, may require physical monitoring due to medical condition or current treatments.

Observation by non-nursing

If the special is being conducted by a non-Nursing staff member please ensure they are orientated to the [Companion Leaflet](#) for non-nursing staff.